

GENERALISED NON-ITCHY RASH

Check if the patient needs urgent attention → 67.

- Check for tick bite (small dark brown/black scab). If tick bite or tick present and headache, fever or body pain, **tick bite fever** likely →24.
- Test for syphilis and HIV → 110.

Syphilis positive

Secondary syphilis likely

Rash often on palms and soles. May have wart-like lesions on genitals and patchy hair loss.



© CDC Public Health Image Library



© University of Cape Town

Treat for early syphilis → 53.

HIV positive

Give routine HIV care
→ 111.

Syphilis and HIV negative

Was patient at risk¹ of HIV in the past 6 weeks?

Yes

No

- Rash may be part of HIV seroconversion illness. Repeat HIV test after 6 weeks.
- Encourage safe sex practices.

Has patient started anticonvulsant, ART, TB medication, co-trimoxazole or TB preventive treatment (TPT) in the past 3 months?

Yes

Consider drug rash →73.

No

Non-specific viral rash likely

- Patient may have fever, headache, lymphadenopathy, muscle pain/body aches.
- Reassure rash will resolve on its own.
- If fever or pain, give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days.

If rash persists ≥ 2 weeks or diagnosis uncertain, discuss/refer.

¹HIV can be transmitted through sexual contact (assault or consensual, burst condom), occupational exposure (sharps injury, splash to eye, mouth, nose or broken skin), human bite, sharing needles, contact with used condom and exposure to blood in sport or at accident scene.